



Non-Standard Management Plans: Obstetrics and Gynaecology Procedure

1. Scope

Site	Operational Area	Applicable to
Armada Kalamunda Group	Obstetrics and Gynaecology	Midwifery and Medical

2. Purpose

This procedure aims for a proper discussion, documentation, informed consent and mutual understanding between the woman and the health care worker when the practice is non-standard and not based on the best current evidence. This procedure promotes shared decision making between health professionals and women presenting for Obstetrics and Gynaecology care, recognising and respecting women's rights to make their own decisions. To do this the health professional in collaboration with the woman should:

- Assess the women as an individual
- Recognise and respect women's rights to make their own informed decisions
- Formulate an agreed management plan (including arranging investigations and providing treatment and advice).
- Document the management plan, consent, and possible risks of the non-standard management of a condition in woman's Handheld Record and Digital Medical Records (DMR). Facilitate coordination and continuity of care should conflict arise between the health professional and woman including the handing over of clinical responsibility to a suitably qualified health professional where possible.

3. Background

In clinical practice, the care and safety of the woman and her baby is our primary concern. Best practice requires all health professionals to:

- Assess the woman, considering the history and woman's views, and perform an appropriate physical examination with appropriate informed consent. The history includes relevant psychological, social, and cultural aspects.
- Refer a woman to another practitioner when this is in the woman's best interest, or the care required extends outside the practitioner's sphere of current practice or another practitioner is better qualified/skilled to provide appropriate care.
- Recognise and respect women's rights to make their own decisions.
- Explain risks and benefits of all alternative procedures and management plans. Document in Handheld Records and Digital Medical records (DMR)
- Refusal of care is an independent risk factor for perinatal mortality

Best practice includes maintaining adequate records, considering the balance of benefit and harm in all clinical decisions, and communicating effectively with women

4. Procedure

If following discussion around best practice and evidence-based care options, a woman indicates a clear choice for care which is outside of health service standard care guidelines, then the health professional must facilitate a second opinion from an alternative, suitably qualified health professional.

- Midwife should refer woman to GP Obstetrician or Consultant Obstetrician.
- GP Obstetrician should refer woman to Consultant Obstetrician.
- Consultant Obstetrician should refer woman to another Consultant Obstetrician.

Should a plan of care requested by the woman have the potential to result in serious harm to the woman and/or fetus, a Consultant Obstetrician referral and review is required.

Informed discussion by health professionals should use the BRAINED acronym as follows:

- **B-Benefits:** *what are the benefits of this test or procedure to me and my baby?*
- **R-Risks:** *What are the risks of this test or procedure to me and my baby?*
- **A-Alternatives:** *Are there any alternatives to this test or procedure?*
- **I-Instinct:** *What do I think about this test or procedure? What feels right about it? Is there someone else I can ask?*
- **N-Nothing:** *What might happen if I choose to do nothing or decide to wait and see?*
- **E-Excuse us:** *Give time for the woman and her partner to think and talk alone.*
- **D-Decision:** *Document her decision and next step.*

If a woman still requests non-standard care after discussion with two health professionals, decision should be recorded on a non-standard management sticker ([Appendix 1](#)) within the Handheld Record and DMR. The substance of the Non-Standard procedure must be recorded in the DMR

- Advise women of recommended guidelines and reasoning/evidence:
 - Ensure risks are neither understated nor overstated
 - Present evidence using absolute risk rather than %
 - Increasing from a 1in 100 to a 2in 1000 risk
 - Rather than increasing from 0.1% to 0.2%
- After a non-standard management sticker is inserted in the case notes, the details of the agreed plan, in consultation with the woman must be documented and the health professional must ensure that documentation is clear and sufficient to facilitate continuity of care.
- If the woman's clinical situation changes, then the non-standard management plan should be discussed again with the woman by the caring team at the time. If changes to the care plan are agreed, then the revised care plan must be clearly documented.
- Develop "if", "then" scenarios and plan for emergencies so that the women's wishes are known.
- If at any time the woman wishes to amend her care plan, any agreed changes must be clearly documented.

- In choosing non-standard care, women are accepting responsibility for outcomes associated with their care choices. Clinical teams have a duty of care to provide information which assists women to make well informed decisions.

5. Follow Up Care:

5.1 Supporting the woman:

- Offer opportunity for woman to discuss her experience at an appropriate time
- Discussion can be deferred to a later date
 - Held over several session
 - Inclusive of support people

5.2 Discussion

- Acknowledge if there were no adverse outcomes from declining care
- Any labour or birth concerns
- Reason for unplanned events of interventions
- Implications for future pregnancies, including how future birth could be planned
- Information on the health services feedback service

5.3 If there is a poor outcome the woman may feel guilty about her decision:

- Actively listen
- Do not engage in judgmental and punitive interactions as it may harm the woman and have lasting negative effects
- Communicate honestly and compassionately with the women
 - *medical decision-making is complex and well-intentioned people can make decisions they later regret, acknowledging that 'you made, what you thought, was the best decision for you and your baby, at the time, in your situation'*
 - *the adverse outcome was not a certainty*
- Open disclosure processes
- Counselling resources for women and family

6. Legislative context

The following documents are required to give effect to this procedure:

- Health Department of Western Australia (HDWA) [Consent to Treatment Policy MP 0175/22](#)
- HDWA [Clinical Handover Policy MP 0095](#)

7. Supporting information

The following documents support the implementation of this procedure:

- [Education – Childbirth and Beyond Guideline OBS-GUI-0144-15](#)
- East Metropolitan Health Service (EMHS) [Consent Policy EMHS: 5](#)

8. Compliance, monitoring and evaluation

Compliance with this policy will be monitored via:

- Monitoring of reported clinical incident data via Datix CIMS.

Specific findings are to be reported at the departmental meeting for review and identification of any opportunities for improvement.

9. Relevant standards

National Safety and Quality Health Service (NSQHS) Standards (second edition):

- Standard 2 – Action 2.4 Healthcare rights and informed consent
- The health service organisation ensures that its informed consent processes comply with legislation and best practice
- Standard 2 – Action 2.5
- The health service organisation has processes to identify:
 - a. The capacity of a patient to make decisions about their own care
 - b. A substitute decision-maker if a patient does not have the capacity to make decisions for themselves
- Standard 5 – Action 5.3 Partnering with consumers
- Clinicians use organisational processes from the Partnering with Consumers Standard when providing comprehensive care to:
 - a. Actively involve patients in their own care
 - b. Meet the patient's information needs
 - c. Share decision-making
- Standard 5 – Action 5.5 Collaboration and teamwork
- The health service organisation has processes to:
 - a. Support multidisciplinary collaboration and teamwork
 - b. Define the roles and responsibilities of each clinician working in a team

10. References

1. Good Medical practice: A Code of Conduct for Doctors in Australia AMC July 2009
2. Obstetric non-standard management plan, FSH reference#FSFH-OBS-POL-003
3. The Royal Australian and New Zealand College of Obstetricians and Gynaecologist (RANZCOG) Maternity suitability for models of care (C-Obs-30)
4. The Australian College of Midwives National Midwifery Guidelines for Consultation and Referral 4th Edition

11. Policy contact (relating to these procedures)

Head of Obstetrics and Gynaecology

Enquiries relating to this procedure may be directed to:

Title: Director Obstetrics and Gynaecology
Department: Maternity
Email: Sangeeta.MallaBhat@health.wa.gov.au

12. Review

This procedure will be reviewed and evaluated as required to ensure relevance and currency. At a minimum it will be reviewed within one (1) year after first issue and at least every three (3) years thereafter.

Version	Effective from	Effective to	Amendment(s)
3	08/03/2024	08/03/2027	Scheduled review

13. Authorisation

This procedure has been authorised and issued by the Director Clinical Services.

Authorised by	Tania Strickland – A/Director of Clinical Services
Authorisation date	08/03/2024
Published date	26/04/2024

Appendix 1: Non-Standard Management Plan

NON-STANDARD MANAGEMENT PLAN	
Patient Name:	
Clinical Problem:	
Management Plan:	
Duration of Plan:	
To be completed if patient's request is contrary to AHS guidelines	
understand that the above plan is not recommended by	
DR/RM	and not supported by evidence based AHS guidelines
<i>Following the above plan may place myself and / or my baby at increased risk</i>	
Doctor / Midwife's Signature	Patient's Signature
Date	Date